

Region 5 Medical Control Authority Network
General Treatment
Ground-level Falls

Initial Date: 7/8/25
Revised Date:

Section 1.11

Purpose: To provide guidance to EMS on appropriate assessment of patients presenting after a ground-level fall without apparent injuries. These patients may have subtle underlying medical conditions contributing to the fall which need further medical evaluation and treatment in the ED.

Protocol:

- I. Primary Assessment
 - A. Follow **General Pre-Hospital Care** protocol
 - B. If injuries present, follow **Adult/Pediatric Trauma Triage** and **General Trauma** protocols
 - C. If no apparent injuries, proceed with MEMAW Assessment
- II. MEMAW Assessment- Conduct assessment using MEMAW

M anual Vitals	Tachycardia (HR > 100) or relative hypotension (SBP < 110) is concerning for sepsis.
E vents leading to fall	Any new symptoms? Generalized weakness? Poor appetite? Shortness of breath? Syncope?
M edications	Any new medications? Blood Thinners? Anti-platelet agents?
A ssessment (including stroke)	Head-to-Toe assessment - not just for trauma, but for medical illness such as stroke, CHF or infection
W alk	ALWAYS determine if able to ambulate at baseline prior to refusal.
... and document what you find!	... involve family and caregivers ... be specific about your concerns when discussing refusal.

- A. Assess manual vital signs: a pulse greater than 100 or less than 60 or new irregularly irregular rhythm, or a systolic blood pressure less than 110 should be considered abnormal
- B. Inquire about events leading to the fall: new or acute symptoms preceding the fall should raise suspicion for a non-mechanical mechanism for the fall and should be considered an abnormal assessment finding
- C. Review patient's current medications: new prescriptions started within the past month, blood thinners, or antiplatelet medications should be considered an abnormal finding
- D. Complete a head-to-toe exam: signs of acute trauma, acute stroke, acute or worsening CHF, infection, or other acute or worsening medical illness should be considered an abnormal finding
- E. Assess walking status: patients unable to demonstrate that they can stand up and ambulate (with their normal walking aid) at their baseline functioning level,

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should be considered an abnormal finding

- F. For patients with abnormal MEMAW assessment findings, refer to the appropriate prehospital protocol for that sign/symptom/condition

III. Symptomatic patients or those with an abnormal MEMAW assessment finding should be transported to the hospital for further evaluation and receive treatment under the appropriate protocol.

- A. Symptoms leading to a fall may be subtle and/or transient and not fully appreciated during brief assessment on scene. Therefore, all patients should be encouraged to seek further care in the Emergency Department.
- B. Transport per **Transport Destination and Diversion** protocol.

IV. Patient refusal

- A. If patient indicates they do not want transport to an Emergency Department, this should be considered a high-risk refusal. Medical control should be consulted if there are any abnormal findings on MEMAW assessment as above.
- B. Follow **Refusal of Care Adult and Minor** protocol

Documentation

- I. Documentation items should include
 - a. Time of call and arrival.
 - b. Patient's current condition versus baseline.
 - c. Statements denying injury or illness.
 - d. Actions taken (e.g., assisted from floor to bed).
 - e. Patient outcome and disposition.
 - f. Any witnesses or family on scene.
 - g. Documentation of high-risk refusal findings listed above.

Reference: Dorsett, M., Allen, H., Garbacz, H., Sensenbach, B., Kumar, S., Jones, C. M. C., & Cushman, J. T. (2025). Timeline for Repeat EMS Encounters Resulting in Transport Following "Lift Assist" in a Suburban EMS System. *Prehospital Emergency Care*, 1–6.
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